3.1 Pneumonia

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I cough a lot but cannot bring up the phlegm." **Objective** • Coughing up thick yellow sputum • Crackles heard over the right lower lobe • RR 26/min	**ACTUAL** **Ineffective airway clearance** related to retained thick secretions as evidenced by crackles, ineffective cough and tachypnea.	Within 48 hours he coughs up secretions effectively and breath sounds are better.	• Listen to the lungs every 4 hours. • Unless fluids are restricted, encourage 2–3 L/day by mouth to loosen the secretions. • Teach controlled coughing and deep breathing. • Give the nebulised bronchodilator or mucolytic as prescribed. • Do chest physiotherapy if ordered. • If he cannot clear the secretions himself, suction him.	Fewer crackles, cough working well: **goal met**. If not, reassess and let the physician know.
Subjective • "I get short of breath when I talk." **Objective** • SpO_2 89% on room air • Using his accessory muscles • Restless	**ACTUAL** **Impaired gas exchange** related to alveolar consolidation as evidenced by hypoxemia, dyspnea and restlessness.	SpO_2 kept at 94% or above within 24 hours.	• Watch SpO_2, RR and level of consciousness. • Sit him in high Fowler, or when he lies on his side, put the good lung down (unaffected side down). • Give oxygen as prescribed and check the device is delivering it properly. • Spread activities out with rest in between. • Check ABG as ordered.	SpO_2 95%, less breathless: **goal met**.
Subjective • Complains of chills **Objective** • Temperature 38.8 °C • WBC 16 × 10^9/L • Age 72 years	**RISK** **Risk for shock** as evidenced by risk factors: infection and possible progression to sepsis.	BP, HR and urine output stay stable.	• Vital signs every 4 hours, more often if he is unstable. • Once cultures are taken, give the antibiotics on time. • Keep a record of urine output. • Report systolic BP below 90 mmHg, HR above 120/min, or any new confusion.	Vital signs stable: **goal met**.